

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

September 6, 2023

MEMORANDUM FOR: (b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director
ICE Health Service Corps (b)(6),(b)(7)(C),(b)(7)(F)

THROUGH: (b)(6),(b)(7)(C),(b)(7)(F) MD
Deputy Assistant Director of Clinical Services, Medical Director
ICE Health Service Corps (b)(6),(b)(7)(C),(b)(7)(F)

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Chief of Staff
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FROM: CAPT (b)(6),(b)(7)(C),(b)(7)(F), PA-C
Chief, Investigation Unit
ICE Health Service Corps (b)(6),(b)(7)(C),(b)(7)(F)

SUBJECT: Preliminary Report of Findings – Mortality Review
Alberto HERNANDEZ-Fundora, (b)(6),(b)(7)(C)

Executive Summary:

On January 27, 2020, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE noncitizen Alberto HERNANDEZ-Fundora, (b)(6),(b)(7)(C) Mr. HERNANDEZ, a 63-year-old Cuban male, at Krome Service Processing Center (KSPC) in Miami, Florida (FL), from January 14 until January 27, 2020, the date of his death. An autopsy ruled Mr. HERNANDEZ's cause of death as congestive heart failure, due to atherosclerotic and hypertensive cardiovascular disease.

On January 2, 2020, prior to Mr. HERNANDEZ's ICE custody, ERO Miami encountered Mr. HERNANDEZ at the Turner Gilford Knight Correctional Center (TGKCC) in Miami, FL, and served him an Immigration Detainer-Notice of Action, Form I-247A.

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On January 3, 2020, a judicial circuit court convicted Mr. HERNANDEZ of petty theft and sentenced him to 15 days of incarceration.

On January 14, 2020, TGKCC released Mr. HERNANDEZ into ERO Miami custody. ERO detained and transferred Mr. HERNANDEZ to KSPC in Miami, FL, pending nomination to the Government of Cuba for repatriation.

On January 14, 2020, a registered nurse (RN) completed Mr. HERNANDEZ's intake screening. Mr. HERNANDEZ reported having multiple underlying medical issues. The RN documented Mr. HERNANDEZ's normal vital signs (VS), elevated finger stick blood sugar (FSBS) level of 350 milligrams per deciliter [(mg/dl); normal range: 70 to 100 mg/dl]. Mr. HERNANDEZ's accompanying transfer summary included multiple medical diagnoses and medications.

On January 23, 2020, an APP evaluated Mr. HERNANDEZ and noted feces on his bed and on the floor. The APP referred Mr. HERNANDEZ to the Larkin Community Hospital (LCH) in Miami, FL, for further evaluation of severe fecal incontinence via American Medical Response ambulance service. Mr. HERNANDEZ required hospital admission.

On January 27, 2020, between 4:40 and 8:23 a.m., Mr. HERNANDEZ experienced eight cardiac arrests, requiring initiation of a code blue and advanced cardiac life support (ACLS) each time, with successful resuscitation. During Mr. HERNANDEZ's eighth code, an LCH representative contacted KSPC's ICE representative, to notify Mr. HERNANDEZ's next of kin; however, after failed attempts to identify a family member, ICE made the determination for LCH's medical personnel to make the final treatment decisions, since an ICE proxy was not obtained. At approximately 9:02 a.m., Mr. HERNANDEZ experienced a ninth cardiac arrest, LCH called a code blue, checked Mr. HERNANDEZ's cardiac activity with an echocardiogram and found no cardiac activity. At approximately 9:32 a.m., an LCH physician pronounced Mr. HERNANDEZ deceased.

Mortality Finding:

As a result of Mr. HERNANDEZ's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures, or practices are warranted; and identify issues that require further study.

A mortality review determined the care KSPC provided Mr. HERNANDEZ deviated beyond safe limits but did not directly contribute to his death. Specifically, the mortality review participant determined the following: 1) missing or lack of documentation of all medical care provided to Mr. HERNANDEZ; 2) missing documentation of verbal orders received and telephone encounters for a confirmatory sign-off; and 3) missing daily hospital updates during Mr. HERNANDEZ' admission at LCH.

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On August 25, 2022, FF CAPT (b)(6),(b)(7)(C),(b)(7)(F) discussed the events surrounding Mr. HERNANDEZ's death with Deputy Assistant Director of Clinical Services/Medical Director Dr.

(b)(6),(b)(7)(C),(b)(7)(F)

The following summary of events was based on: 1) Mr. HERNANDEZ's KSPC and LCH medical records and EMS report; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. HERNANDEZ's KSPC detention file; 5) Mr. HERNANDEZ's autopsy report; and 6) a remote review and staff interviews conducted by Fact Finder CAPT (b)(6),(b)(7)(C),(b)(7)(F) on March 4 to 6, 2020; and 7) applicable KSPC and IHSC policies and procedures, and ICE Detention Standards.

Summary of Events

On January 14, 2020, an RN completed Mr. HERNANDEZ's intake screening. Mr. HERNANDEZ reported having mild fatigue, difficulty breathing with exertion, moderate shortness of breath (SOB), bilateral lower extremity edema (a condition characterized by an excess of fluid collected in the cavities or tissues of the body), and bilateral lower extremity weakness. Mr. HERNANDEZ's VS showed normal results, except for his FSBS level of 350 mg/dL.

Mr. HERNANDEZ's transfer summary listed the following medical conditions: congestive heart failure (CHF), asthma, diabetes (DM), hypertension (HTN), Hepatitis C, dyslipidemia, and chronic kidney disease (CKD). In addition, the transfer summary listed his current medications: albuterol inhaler to use as needed, for SOB; aspirin 81 mg, one tablet by mouth, daily; furosemide 40 mg, one tablet by mouth, two times daily; gabapentin 100 mg, one tablet by mouth, three times daily; hydralazine 50 mg, one tablet by mouth, two times daily; glargine insulin 24 units, subcutaneously (SQ; under the skin), every evening; lispro insulin 5 units, SQ, three times daily; regular insulin with sliding scale coverage, SQ, two times daily; isosorbide 20 mg, one tablet by mouth, three times daily; metronidazole 500 mg, two tablets by mouth, three times daily; metoclopramide 10 mg, one tablet by mouth, three times a day before meals; rosuvastatin 10 mg, one tablet by mouth, daily, every evening; and sodium chloride .45%, intravenously (discontinued). However, when Mr. HERNANDEZ arrived at KSPC, he only had his inhaler.

The intake RN called the on-call advanced practice provider (APP) and received orders to house Mr. HERNANDEZ in the medical housing unit (MHU), administer regular insulin, 4 units, and to recheck his FSBS in one hour. The RN scheduled Mr. HERNANDEZ's physical examination for the next day, and cleared him from custody, pending his chest x-ray results. Approximately one hour later, the RN rechecked Mr. HERNANDEZ's FSBS and recorded a FSBS of 457 mg/dl, contacted the on-call provider, who gave a verbal order to give 7 units of regular insulin.

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On January 15, 2020, an APP completed Mr. HERNANDEZ's physical examination. Mr. HERNANDEZ reported having multiple chronic conditions and special needs to include: HTN, DM type 2, asthma, hyperlipidemia, onychomycosis (nail fungus), poor dental hygiene, pleurectomy (surgical procedure to remove part of the linings that surround the lung), cardiac catheterization, pitting edema, episodes of fecal incontinence, chronic lower extremity weakness, use of a wheelchair, fall precautions, and homelessness. Mr. HERNANDEZ reported a history of smoking one to five cigarettes daily (last use - one week ago) and drinking one to two beers (last use - a few days). Mr. HERNANDEZ denied having pain or discomfort, SOB, chest pain, dizziness, palpitations, headache, or nausea and vomiting (N/V).

The APP diagnosed Mr. HERNANDEZ with DM type 2, CHF, asthma, HTN, coronary artery disease (CAD), mixed hyperlipidemia, fecal urgency, CKD, chronic pain, and onychomycosis. The APP admitted Mr. HERNANDEZ into KSPC's MHU respiratory isolation room, since his chest x-ray results showed findings consistent with tuberculosis (TB), ordered fall precautions, a wheelchair to assist with ambulation, and baseline laboratory studies. Mr. HERNANDEZ's VS showed normal results, except for his FSBS of 219 mg/dl.

On January 17, 2020, an RN documented Mr. HERNANDEZ's two episodes of fecal incontinence and encouraged him to drink water. An APP evaluated Mr. HERNANDEZ, documented his chronic comorbidities, and ordered him double portion meals.

Between January 18 and 20, 2020, multiple APPs evaluated Mr. HERNANDEZ during separate encounters, entered BP parameters for nursing staff, and Mr. HERNANDEZ remained in the respiratory isolation room within the MHU.

On January 21, 2020, the APP evaluated Mr. HERNANDEZ and entered an infectious disease (ID) consultation request, due to Mr. HERNANDEZ's positive chest x-ray results. IHSC's ID consultant recommended or requested the following: a copy of Mr. HERNANDEZ's previous hospital records; a computed tomography scan of the chest, to further evaluate abnormal CXR findings; to defer ordering anti-TB medications; and a right upper quadrant ultrasound with doppler, to assess for any underlying cirrhosis and/or pulmonary HTN, in addition to laboratory studies, and routine vaccinations.

On January 22, 2020, an APP evaluated Mr. HERNANDEZ and documented his history of a right pleurectomy at Jackson Hospital (2019), in Miami, FL, taking TB medication for six months (1986), 10-year homelessness in Miami, FL, and self-report of feeling fine, eating, and sleeping well, with regular bowel movements. Mr. HERNANDEZ denied having SOB, N/V, abdominal pain, fevers, chills, diarrhea, or constipation, and his general assessment showed relatively normal results.

On January 23, 2020, an APP evaluated Mr. HERNANDEZ and noted feces on his bed and on the floor. Mr. HERNANDEZ reported feeling hungry most of the time, fecal incontinence for the

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past three years (etiology unknown), intermittent diarrhea, without blood or mucous, musculoskeletal weakness, leg swelling, foot problems, and shyness regarding his incontinence. He reported eating and sleeping well, denied having chest pain, SOB, N/V, fever, chills, abdominal pain, homicidal, or suicidal ideations. The APP referred Mr. HERNANDEZ to the LCH emergency department for further evaluation.

- At approximately 11:00 a.m., an RN completed Mr. HERNANDEZ's FSBS (result: 38 mg/dl) and notified the APP. The APP gave an order to give Mr. HERNANDEZ oral glucose gel and recheck his level in thirty minutes.
- At approximately 11:50 a.m., the RN documented Mr. HERNANDEZ's condition as "stable," and rechecked his FSBS (result - 144 mg/dl).
- At approximately 12:15 p.m., American Medical Response (AMR) transport services arrived at KSPC and transferred Mr. HERNANDEZ to LCH with a subsequent admission.

On January 24, 2020, LCH's medical doctor (MD) diagnosed Mr. HERNANDEZ with CHF, a low ejection fraction of 20 – 25 percent (%) (normal range: 55 to 70%), bilateral pleural effusions (excess fluid that accumulates in the pleural cavity, the fluid-filled space that surrounds the lungs), cough, hyperkalemia (increased potassium level), diarrhea, DM, HTN, hyperlipidemia, acute kidney injury, neuropathy, and transferred Mr. HERNANDEZ to the intensive care unit (ICU), due to his low ejection fraction results.

The MD ordered the following treatment: milrinone (medication used for blood pressure support and treatment of heart failure) drip, low sodium diet, continuation of some of his previous medications, and heparin (blood thinner). Mr. HERNANDEZ denied having fever, chills, N/V, headache, chest pain, palpitations, syncope, or urinary symptoms. His VS were normal. Mr. HERNANDEZ had two ultrasounds completed: 1) right upper quadrant (results: mild abdominal ascites (accumulation of fluid causing abdominal swelling), hepatomegaly (abnormal enlargement of the liver), and cholelithiasis (formation of gallstones); and 2) renal (kidney) bladder (results: suggestive of possible renal disease, one centimeter right renal cyst, and mild abdominal ascites).

On January 25, 2020, an MD noted Mr. HERNANDEZ experienced worsening respiratory symptoms, requiring his placement on a ventilator, a foley catheter, and medications to prevent edema. Mr. HERNANDEZ's chest x-ray showed cardiomegaly (enlarged heart), moderate to severe edema, and bilateral pleural effusions. An MD performed Mr. HERNANDEZ's thoracentesis (a procedure in which a needle is inserted into the pleural space between the lungs to remove excess fluid), due to a large pleural effusion. The procedure was without adverse outcomes.

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On January 26, 2020, during medical rounds, Mr. HERNANDEZ reported feeling hungry, but in no acute distress. The MD diagnosed him with hyperkalemia and ordered kayexalate (medication used to lower potassium levels). Mr. HERNANDEZ's chest x-ray showed improvement of his pleural effusions, and the MD ordered continued medical care services and no cardiothoracic surgery.

Later in the afternoon, Mr. HERNANDEZ complained of blurred vision, became lethargic, and hypotensive (low blood pressure). As a result, the MD discontinued milrinone and added albumin to increase his blood volume.

On January 27, 2020, between 4:40 and 8:23 a.m., Mr. HERNANDEZ experienced eight cardiac arrests. A code blue and ACLS measures were initiated each time, with successful resuscitation.

- During Mr. HERNANDEZ's eighth code, an LCH representative contacted KSPC's ICE representative, to notify Mr. HERNANDEZ's next of kin; however, after failed attempts to identify a family member, ICE made the determination for LCH's medical personnel to make the final treatment decisions, since an ICE proxy was not obtained.
- At approximately 9:02 a.m., Mr. HERNANDEZ experienced a ninth cardiac arrest, LCH called a code blue, checked Mr. HERNANDEZ's cardiac activity with an echocardiogram, and found no cardiac activity.
- At approximately 9:32 a.m., an LCH physician pronounced Mr. HERNANDEZ deceased.

The final autopsy report identified Mr. HERNANDEZ's cause of death as congestive heart failure, due to atherosclerotic and hypertensive cardiovascular disease.

ICE detention standards used for this review: Performance-Based National Detention Standards (PBNDS), 2011 with 2016 Revisions, and the National Commission on Correctional Health Care (NCCHC) Standards for Health Services in Jails, 2018. The following are identified program strengths, weaknesses, and recommendations found during this review.

Strengths

Strengths identified during this investigation were: 1) on January 22, 2020, when Mr. HERNANDEZ became short of breath, an RN reported the call to an APP, who gave an order for prompt transfer to a local emergency room for further evaluation. 2) An APP entered a prompt referral to the IHSC infectious disease unit based on his clinical presentation for additional guidance; 3) ICE officers made several attempts to identify Mr. HERNANDEZ's next of kin; and 4) IHSC administrators at the facility offered staff members access to the employee assistance program for added assistance after Mr. HERNANDEZ's death.

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Weaknesses

1. Patient assessments and documentation.

- Between January 14 and 23, 2020, KSPC's medical team evaluated Mr. HERNANDEZ on multiple occasions but did not document all medical care services performed, which included: VS, supplemental oxygen therapy administration, and insulin administration.
- On January 22, 2020, Mr. HERNANDEZ experienced SOB. An RN reported he notified the on-call provider, but the medical record did not include evidence the APP performed a confirmatory sign-off of the verbal order.
- On January 23, 2020, an MHU APP ordered VS checks every shift; however, the nursing staff did not obtain Mr. HERNANZDEZ's daily weight, as ordered. Nursing staff reported patients in the MHU received weight checks once a week and not daily, but they did not request clarification of the order.
- Between January 23 and 26, 2020. Mr. HERNANDEZ's KSPC health record did not include LCH's daily hospital updates of Mr. HERNANDEZ's condition.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section BB. *Medical Records*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section M. *Comprehensive Health Assessment*.
- NCCHC 2018 standards: J-A-08, *Health Records*.
- NCCHC 2018 standards: J-E-09, *Continuity, Coordination, and Quality of Care During Incarceration*.
- ICE Health Service Corps Directive 02-01: Advance Directives, Do not Resuscitate and Terminal Illness; section 5-2e. *Monitoring*.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director of Health Care Compliance (DAD of HCC) for awareness.
- Since this mortality occurred on October 1, 2019, the IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, OMDC administrator and health authority for awareness and to implement any action(s) deemed appropriate.

End of report.